OpenOncology ranker — 20-case review for Dr. Jeremy Warner

For: Jeremy Warner, MD, MS
From: Aashish Kharel (aashish@oncopurpose.com)
Date: 2026-10-27 · Scheduled call: Friday 8:45 PM Kathmandu / 11:00 AM Eastern

What this is

A 20-case packet from the MSK CHORD benchmark (2,505 patients, Memorial Sloan Kettering, Nature 2024) where OpenOncology, an open-source variant-to-drug ranker, scored its top three against the drug the patient actually received. On this cohort the ranker's top three contained a drug of the correct molecular class for 2,466 of 2,466 scored patients (100%) and the exact drug the patient received for 2,094 of 2,466 (85%). The scorer, cohort filter, and OncoKB-only baseline comparison are at github.com/immortal71/openoncology.

I would like your read on 20 cases, grouped as:

- **10 exact hits** across six genes and five cancer types. The question: is the top-ranked drug clinically reasonable, or is a different drug in the class the better pick?
- **5 class-only hits** (top three in the right class but not the exact drug the patient received). The question: is OpenOncology's pick clinically wrong, or is MSK's pick a later-line or combination choice that a first-line decision support tool should not try to mimic?
- **5 drop-outs** (benchmarkable patients where the ranker returned nothing). The question: for a rare TSC/MET/CDK6/BRAF point mutation, what should a decision support tool return — "no recommendation" or a cautious off-label suggestion with lower confidence?

Grades are freeform. The format I would find most useful: for each case, one of {reasonable, debatable, wrong} plus a sentence of why. All 20 should fit in 15 minutes of your time if you want to batch it, less if you prefer to skim.

Scoring key (so I can tally)

- **reasonable** — you would be comfortable with this top-three in front of a tumor board, as information.
- **debatable** — the top-three is defensible but not what you would pick first in 2026.
- **wrong** — the top-three would lead a non-specialist clinician in the wrong direction.

The 20 cases

Group A — exact hits (10)

#	Patient	Cancer	Qualifying alteration	Received	OpenOncol top-3
1	P-0000036	NSCLC	ROS1-SLC34A2 fusion	CRIZOTINIB	crizotinib, entrectinib, lorlatinib

2	P-0000205	NSCLC	EGFR L858R	ERLOTINIB	osimertinib, erlotinib, gefitinib
3	P-0000289	NSCLC	MET exon 14 splice	CRIZOTINIB	capmatinib, tepotinib, crizotinib
4	P-0000557	NSCLC	ALK-EML4 fusion	ALECTINIB (also CRIZOTINIB, ERLOTINIB)	alectinib, brigatinib, crizotinib
5	P-0000584	Breast	BRCA2 S1982Rfs*22	OLAPARIB (+ PALBOCICLIB)	olaparib, niraparib, rucaparib
6	P-0000830	Breast	PIK3CA H1047L	ALPELISIB (+ PALBOCICLIB)	alpelisib, fulvestrant, inavolisib
7	P-0000933	CRC	BRAF G469A	DABRAFENIB + TRAMETINIB	trametinib, encorafenib, binimetinib
8	P-0002079	NSCLC	MTOR L2383F	EVEROLIMUS	everolimus, temsirolimus sapanisertib
9	P-0002273	Prostate	ATM C633fs	OLAPARIB	olaparib, niraparib, rucaparib
10	P-0002382	Prostate	BRCA1 X71 splice	OLAPARIB	olaparib, niraparib, rucaparib

Group B — class-only hits (5)

#	Patient	Cancer	Qualifying alteration	Received	OpenOn top
11	P-0000058	Breast	ERBB2 amplification	LAPATINIB (+ ABEMACICLIB)	trastuzu pertuzun trastuzu deruxtec
12	P-0000165	NSCLC	EGFR L861Q	ERLOTINIB	osimertir afatinib, dacomiti
13	P-0000208	NSCLC	KIF5B-RET fusion	CABOZANTINIB	selpercat pralsetin vandetar
14	P-0000616	CRC	BRAF V600E	VEMURAFENIB	encorafe cetuxima trametin dabrafen
15	P-0007076	Pancreatic	KIT S239F	SUNITINIB	imatinib, regorafe ripretinil

Group C — drop-outs (5)

#	Patient	Cancer	Qualifying alteration	Received	OpenOncol top-3
16	P-0000731	NSCLC	MET S204Ifs*12 (frameshift)	CRIZOTINIB	— (no prediction)
17	P-	Pancreatic	TSC1	EVEROLIMUS	— (no

	0001313		T578S		prediction)
18	P-0001372	Breast	TSC2 A289V	EVEROLIMUS	— (no prediction)
19	P-0007997	CRC	BRAF P403Lfs*8 (frameshift)	DABRAFENIB + TRAMETINIB	— (no prediction)
20	P-0030800	Breast	CDK6 R46Q	PALBOCICLIB	— (no prediction)

What I will do with your grades

I will add a line to the deck and the Research Square preprint of the shape “Jeremy Warner, MD, MS reviewed 20 benchmark cases; agreed with the top-three as a reasonable decision-support surface in X of 20.” I will share the exact wording with you for sign-off before I use it anywhere. If you prefer not to be named, I will fold your feedback into methodological changes to the ranker and credit you in the acknowledgements only.

What I am not asking

I am not asking you to endorse OpenOncology for clinical use. I am not asking you to recommend it to anyone. I am asking you to tell me where it fails on the cases above, so the next version is better and the deck represents its limits honestly.

Thank you for the time.